This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Pneumovax 23 or Prevenar 20 to vaccinate against Pneumococcal Disease

Patient Group Direction, version 009, issued 30 September 2026. Supersedes Version 008, 24 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for pneumococcal vaccination: Green Book chapter 25 (Pneumococcal), edition of 29 July 2026, the JCVI advice of June 2023 that PPV23 or PCV20 may be used in the adult and at-risk programmes, the JCVI advice of June 2024 on people experiencing homelessness, and the Prevenar 20 and Pneumovax 23 SmPCs. NICE CKS 'Immunizations - pneumococcal' summarises the same programme.

Note: this summary describes the national programme as set out in the July 2026 Green Book chapter, which moved adults at 65 and the clinical risk groups from PPV23 to Prevenar 20 (PCV20) in early 2026 and includes the infant schedule. The products authorised under this PGD are Prevenar 20 and Pneumovax 23, for individuals aged 2 years and over. Where Prevenar 20 is held it is given in preference; Pneumovax 23 is given where Prevenar 20 is not held, where the subcutaneous route is needed, and for the later 5-yearly revaccination cycles of asplenia, splenic dysfunction and chronic kidney disease once Prevenar 20 has been given once. The infant schedule, and the multi-dose sequences for severe immunocompromise and bone marrow transplant, are outside this PGD: refer.

Overview

- Pneumococcal disease is caused by *Streptococcus pneumoniae*, leading to infections such as **pneumonia**, **septicaemia**, and **meningitis**.
- Vaccination is aimed at **preventing invasive pneumococcal disease (IPD)**, especially in high-risk groups.

Pneumococcal Vaccines Available in the UK (Green Book Table 1)

1. **Pneumococcal conjugate vaccines (PCV): Prevenar 13 (PCV13), Vaxneuvance (PCV15), Prevenar 20 (PCV20) and Capvaxive (PCV21):**
    - **Prevenar 13 is the routine infant vaccine. Prevenar 20 has been used in the adult (65 years) and at-risk programmes since early 2026 and replaces PPV23 as national stock. Vaxneuvance (from 6 weeks) and Capvaxive (from 2 years, approved by the MHRA in 2026) are licensed but not in the national programme.**
2. **Pneumococcal polysaccharide vaccine (PPV23, Pneumovax 23):**
    - **Protects against 23 serotypes. Children under 2 years respond poorly and there is no evidence of effectiveness below that age.**
    - Recommended for risk groups since 1992 and for everyone aged 65 and over since 2003. PPV23 is no longer available to order through the national programme; locally held stocks are used up first, then PCV20.

Routine Childhood Schedule (UK)

- **PCV13 (Prevenar 13) is given as:**
  - **A single priming dose at 16 weeks of age (moved from 12 weeks on 1 July 2025 so that the second MenB dose can be given at 12 weeks)**
  - **A booster dose at 1 year of age (on or after the first birthday), with the other vaccines due at that age**

Doses of any PCV given before 12 weeks of age are discounted. An unimmunised or partially immunised child aged 1 to under 2 years has a single dose of PCV13; routine PCV is not offered after the second birthday. The infant schedule is outside this PGD.

Clinical Risk Groups (Green Book Table 2)

Offer a single dose of PPV23 or PCV20 to individuals aged 2 years and over with:

- **Asplenia or splenic dysfunction (including homozygous sickle cell disease, hereditary spherocytosis, thalassaemia major and coeliac disease with splenic dysfunction); chronic respiratory disease (COPD, bronchiectasis, cystic fibrosis, interstitial lung fibrosis, pneumoconiosis, bronchopulmonary dysplasia, and neurological or neuromuscular disease that compromises respiratory function; asthma only where it needs continuous or frequently repeated systemic steroids); chronic heart disease; chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant); chronic liver disease (cirrhosis, biliary atresia, chronic hepatitis)**
- **Diabetes mellitus requiring insulin or anti-diabetic medication (not diabetes that is diet controlled only)**
- **Immunosuppression due to disease or treatment, including chemotherapy, bone marrow transplant, asplenia or splenic dysfunction, complement disorder, HIV infection at all stages, multiple myeloma, genetic disorders affecting the immune system (IRAK-4, NEMO), and systemic steroids for more than a month at a dose equivalent to prednisolone 20mg or more a day (or 1mg/kg a day in a child under 20kg). Those with severe immunocompromise (bone marrow transplant, leukaemia, myeloma, genetic immune disorders) follow the multi-dose sequence in the schedule table below rather than a single dose**
- **Cochlear implants (immunisation must not delay the implantation)**
- **Cerebrospinal fluid leaks following trauma or major skull surgery (not CSF shunts)**
- **Occupational risk from frequent or continuous exposure to metal fumes, such as welders (PPV23, PCV20 or PCV21, one dose if none given before; Capvaxive (PCV21) is not authorised under this PGD); and people experiencing homelessness (rough sleepers and those using homeless hostels or night shelters, JCVI June 2024)**

Children under 2 years in a clinical risk group receive additional PCV doses on the Green Book Table 3 schedule (PCV20 for asplenia, splenic dysfunction, complement disorder or severe immunocompromise); this is outside this PGD.

Older Adults

- **All adults are offered a single dose of PPV23 or PCV20 at 65 years of age. An individual who has already received PPV23 or PCV20 because they are in a clinical risk group does not need another dose at 65, whatever the interval. Revaccination every 5 years with PPV23 or PCV20 is recommended only for asplenia, splenic dysfunction (including sickle cell disease) and chronic kidney disease; it is not recommended for any other risk or age group. Antibody testing before vaccination is not required.**

Vaccination Schedule by Group (Green Book chapter 25, July 2026, simplified)

Group	Vaccine	Notes
Infants	**PCV13 at 16 weeks and a booster at 1 year (on or after the first birthday)**	Routine; outside this PGD
Adults aged 65 and over	**PPV23 or PCV20, single dose**	Even if healthy. No dose at 65 where PPV23 or PCV20 was already given for a risk group
Clinical risk groups aged 2 years and over (other than the next row)	**One PPV23 or one PCV20**	No further PCV13 is needed from 2 years of age, whatever the earlier PCV history (Green Book Table 3), and Prevenar 13 is not given under this PGD. Repeat every 5 years only for asplenia, splenic dysfunction or chronic kidney disease
Asplenia, splenic dysfunction, complement disorder or severe immunocompromise, aged 2 years and over	**Asplenia, splenic dysfunction or complement disorder: one PPV23 or PCV20. Severe immunocompromise (bone marrow transplant, acute or chronic leukaemia, multiple myeloma, genetic immune disorders): one dose of PCV20 whatever the previous PCV history, then PPV23 or PCV20 at least 4 weeks later; bone marrow transplant recipients have two PCV20 doses at least 4 weeks apart**	Asplenia, splenic dysfunction or complement disorder: one Prevenar 20 (or one Pneumovax 23) under this PGD; asplenia and splenic dysfunction are then revaccinated every 5 years (Prevenar 20 once, Pneumovax 23 thereafter). Severe immunocompromise and bone marrow transplant need specialist input and their multi-dose sequences are not given under this PGD: refer

Contraindications

- Confirmed anaphylactic reaction to a previous dose of a pneumococcal vaccine, or to any component or residue from the manufacturing process. When in doubt seek specialist advice rather than withholding vaccine.
- Acute illness: postpone until fully recovered. Minor illness without fever or systemic upset is not a reason to postpone.
- Pregnancy and breastfeeding: pneumococcal vaccines may be given in pregnancy when protection is needed without delay; there is no evidence of risk from inactivated vaccines in pregnancy or breastfeeding.
- Timing around splenectomy or immunosuppressive treatment: vaccinate ideally 4 to 6 weeks, and at least 2 weeks, before elective splenectomy, chemotherapy or radiotherapy; where that is not possible, vaccinate at least 2 weeks after splenectomy or at least 3 months after completing chemotherapy or radiotherapy (leukaemia: 6 months after chemotherapy; bone marrow transplant: from 9 to 12 months after transplant). Treatment is never delayed for vaccination, and vaccination is not delayed where that would mean it never happens.

Side Effects

- PPV23: mild soreness and induration at the injection site for 1 to 3 days, less commonly a low-grade fever; reactions are commoner in people with high existing antibody levels. PCV13 (children): injection site reactions, fever, irritability, decreased appetite, sleep changes. PCV20 (adults): injection site pain, muscle pain, fatigue, headache, joint pain.
- Rare: allergic reaction including anaphylaxis.

Co-Administration

- Pneumococcal vaccines can be given at the same time as any other vaccine (including influenza and COVID-19) at a separate site, preferably a different limb, or at least 2.5cm apart in the same limb. Record the site of each vaccine.

Documentation

- Record the vaccine name and brand, batch number, expiry date, site, date and route.
- Report adverse reactions via the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Pneumovax 23

for vaccination against

Pneumococcal Disease

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Pneumovax 23 is indicated for active immunisation against pneumococcal disease caused by the 23 serotypes included in the vaccine for individuals aged 2 years and over at increased risk, in accordance with national guidance.
Inclusion criteria	- Individuals aged 2 years and over who are eligible for pneumococcal vaccination under Green Book chapter 25: aged 65 and over; or aged 2 years and over and in a clinical risk group (Green Book Table 2), with occupational exposure to metal fumes (such as welders), or experiencing homelessness. - Where Prevenar 20 is held it is given in preference under its arm of this PGD. Pneumovax 23 is given where Prevenar 20 is not held, where the subcutaneous route is needed, or for a 5-yearly revaccination of asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) in someone who has already had Prevenar 20 (a repeat Prevenar 20 is not authorised). - Where a pneumococcal conjugate vaccine was given, at least 8 weeks have elapsed. - Informed consent obtained from the individual, or for a child under 16 from a person with parental responsibility or from the young person where Gillick competent (see the consent page of this PGD). - No contraindications to pneumococcal vaccination.
Exclusion criteria	- Known hypersensitivity to Pneumovax 23 or any of its components. - Previous severe allergic reaction to any pneumococcal vaccine. - Aged under 2 years. - Not in an eligible group: for example a healthy adult under 65 asking for the vaccine privately. Not authorised under this PGD, whatever the product licence says. - Any previous dose of PPV23, PCV20, Vaxneuvance (PCV15) or Capvaxive (PCV21) given at 2 years of age or older, whatever the interval, except for asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) where at least 5 years have elapsed since the last PPV23 or PCV20 dose (5-yearly revaccination, Green Book chapter 25). An individual vaccinated for a risk group does not need another dose at 65. - Any pneumococcal conjugate vaccine received within the last 8 weeks. - Severe immunocompromise as defined in Green Book chapter 25 (bone marrow transplant, acute or chronic leukaemia, multiple myeloma, or a genetic immune disorder such as IRAK-4 or NEMO deficiency): the multi-dose sequence needs specialist input and is not given under this PGD. Refer. - Currently receiving chemotherapy or radiotherapy, or within 3 months of completing it (6 months after chemotherapy for leukaemia): not given under this PGD; refer to the treating team, which decides the timing

	(part 2: vaccination is not delayed where that would mean it never happens, so long-term maintenance treatment is not an indefinite deferral). Where treatment is planned and has not started, give at least 2 weeks before it does. - Acute illness with fever (postpone until recovered; a minor illness without fever or systemic upset is not a reason to postpone).
Cautions	- Pregnancy: not an exclusion where the woman is in an eligible group; the vaccine is inactivated and the Green Book finds no evidence of risk. Breastfeeding is not a reason to withhold. - Bleeding disorder, thrombocytopenia or anticoagulation: give intramuscularly with a 23 gauge or finer needle and firm pressure without rubbing for at least 2 minutes, or subcutaneously where the haematology team advises against intramuscular injection. - Immunosuppression other than the severe group in the exclusions: the response may be reduced but the vaccine is given; where chemotherapy, radiotherapy or splenectomy is planned, give at least 2 weeks before it starts (part 2). - Cochlear implant planned: vaccinate before implantation without delaying the operation. - Co-administration: may be given at the same time as any other vaccine at a separate site. - Observe the patient for 15 minutes after vaccination. Adrenaline and the anaphylaxis requirements of this PGD apply. - Record the batch number, expiry date and site of administration in the patient record. - Advise on expected side effects and when to seek medical advice.
Actions if patient is excluded or declines treatment	Explain the reason. Where Prevenar 20 is indicated and held, give it under its arm of this PGD; otherwise advise how the vaccine can be accessed through the GP. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Pneumovax 23
Legal category	POM
Storage	Store in a refrigerator (2 to 8 C) in the original packaging to protect from light. Do not freeze; discard if frozen. Apply the cold chain page of this PGD to any excursion.
Route/method of administration	Intramuscular injection into the deltoid, or subcutaneous injection where the intramuscular route is not appropriate (for example on the advice of a haematology team in a bleeding disorder).
Dose and frequency	Single 0.5 mL dose. Revaccination every 5 years ONLY for asplenia, splenic dysfunction or chronic kidney disease (Green Book chapter 25),

	counted from the last PPV23 or PCV20 dose; not recommended for any other group.
Quantity to be administered and/or supplied	0.5 mL per dose.
Maximum or minimum treatment period	A single dose under this PGD. A 5-yearly repeat for asplenia, splenic dysfunction or chronic kidney disease is a further single dose under this PGD; no other group is revaccinated.
Adverse effects	Injection site pain, redness, swelling, fever, fatigue, headache. Rare: allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date, product name and brand, batch number, expiry date, dose, route and site; the eligibility group relied on and the pneumococcal vaccine history checked ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Seek medical advice for a severe or persistent injection site reaction, a fever that does not settle within 48 hours, or any sign of an allergic reaction. Call 999 for difficulty breathing, swelling of the face or throat, or collapse. Adults at 65 and most risk groups need no further pneumococcal vaccine; asplenia, splenic dysfunction and chronic kidney disease have a repeat dose every 5 years.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 25 Pneumococcal, edition of 29 July 2026; JCVI advice on the pneumococcal programme, June 2023 and June 2024; Summaries of Product Characteristics for Prevenar 20 (emc 13461) and Pneumovax 23, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Patient Group Direction

for the supply/administration of

Prevenar 20

for vaccination against

Pneumococcal Disease

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Prevenar 20 (pneumococcal polysaccharide conjugate vaccine, 20-valent, adsorbed) is licensed for active immunisation for the prevention of pneumococcal disease caused by Streptococcus pneumoniae in individuals from 6 weeks of age. Under this PGD it is given to individuals aged 2 years and over who are eligible under Green Book chapter 25: adults aged 65 and over, and individuals aged 2 years and over in a clinical risk group (Green Book Table 2), with occupational exposure to metal fumes, or experiencing homelessness.
Inclusion criteria	- Individuals aged 2 years and over who are eligible for pneumococcal vaccination under Green Book chapter 25: aged 65 and over; or aged 2 years and over and in a clinical risk group (Green Book Table 2), with occupational exposure to metal fumes (such as welders), or experiencing homelessness. - No previous dose of PPV23, PCV20, Vaxneuvance (PCV15) or Capvaxive (PCV21) given at 2 years of age or older (see the exclusion criteria), except that an individual with asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) who is due a 5-yearly revaccination (at least 5 years since the last PPV23) and has never had Prevenar 20 receives Prevenar 20 as that revaccination dose; later cycles are Pneumovax 23. A previous Prevenar 13, at any age, does not exclude, provided at least 8 weeks have elapsed. A child who had Prevenar 20 under 2 years on the risk group schedule is referred (see the exclusion criteria). - Informed consent obtained from the individual, or for a child under 16 from a person with parental responsibility or from the young person where Gillick competent (see the consent page of this PGD). - No contraindications to Prevenar 20.
Exclusion criteria	- Known hypersensitivity to the active substances, to any excipient (sodium chloride, succinic acid, polysorbate 80, aluminium phosphate) or to diphtheria toxoid (CRM197 carrier protein). - Severe allergic reaction to a previous dose of any pneumococcal vaccine. - Aged under 2 years. The infant and toddler schedules are not given under this PGD. - Not in an eligible group: for example a healthy adult under 65 asking for the vaccine privately. Not authorised under this PGD, whatever the product licence says. - Any previous dose of Prevenar 20 (PCV20) given at 2 years of age or older, whatever the interval: under this PGD Prevenar 20 is a single lifetime dose and is not repeated (a repeat dose is not established in the SmPC). Later 5-yearly revaccination of asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) is given with Pneumovax 23 under its arm where held, or through the GP. A child aged 2 to 4 who had Prevenar 20 on the

	under-2 risk group schedule: refer to the GP to complete Green Book Table 3. - Any previous dose of PPV23, Vaxneuvance (PCV15) or Capvaxive (PCV21) given at 2 years of age or older, whatever the interval, except for asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) where at least 5 years have elapsed since the last PPV23 dose (5-yearly revaccination, Green Book chapter 25). An individual vaccinated for a risk group does not need another dose at 65. - Any pneumococcal conjugate vaccine received within the last 8 weeks. - Severe immunocompromise as defined in Green Book chapter 25 (bone marrow transplant, acute or chronic leukaemia, multiple myeloma, or a genetic immune disorder such as IRAK-4 or NEMO deficiency): the multi-dose sequence needs specialist input and is not given under this PGD. Refer. - Currently receiving chemotherapy or radiotherapy, or within 3 months of completing it (6 months after chemotherapy for leukaemia): not given under this PGD; refer to the treating team, which decides the timing (part 2: vaccination is not delayed where that would mean it never happens, so long-term maintenance treatment is not an indefinite deferral). Where treatment is planned and has not started, give at least 2 weeks before it does. - Acute illness with fever (postpone until recovered; a minor illness without fever or systemic upset is not a reason to postpone).
Cautions	- Pregnancy: not an exclusion where the woman is in an eligible group. The vaccine is inactivated and the Green Book finds no evidence of risk from inactivated vaccines in pregnancy; the SmPC has no data in pregnancy and advises use where the potential benefit outweighs any risk. Tell her that, record it, and give the vaccine. Breastfeeding is not a reason to withhold. - Bleeding disorder, thrombocytopenia or anticoagulation: Prevenar 20 is intramuscular only. Give with a 23 gauge or finer needle and firm pressure without rubbing for at least 2 minutes (Green Book chapter 4); on warfarin, confirm the latest INR is up to date and below the top of the target range. Where the individual's haematology team has advised against intramuscular injection, give Pneumovax 23 subcutaneously under its arm of this PGD where held and indicated, or refer. - Immunosuppression other than the severe group in the exclusions (for example HIV, systemic steroids, completed chemotherapy): the response may be reduced but the vaccine is given. Where chemotherapy, radiotherapy or splenectomy is planned, give at least 2 weeks before it starts (part 2). - Cochlear implant planned: vaccinate before implantation without delaying the operation. - Co-administration: may be given at the same time as any other vaccine, including influenza, COVID-19, shingles and RSV vaccines, at a separate

	site (Green Book). The SmPC notes that with adjuvanted influenza vaccine a gap of about 4 weeks may be considered in those at the highest risk of life-threatening pneumococcal disease; this is not a requirement and must not delay either vaccine. - Polysorbate 80 may cause hypersensitivity reactions. - Observe the patient for 15 minutes after vaccination. Adrenaline and the anaphylaxis requirements of this PGD apply. - Record the batch number, expiry date and site of administration in the patient record. - Advise on expected side effects and when to seek medical advice.
Actions if patient is excluded or declines treatment	Explain the reason. Where Pneumovax 23 is indicated and held (subcutaneous route needed, or a 5-yearly revaccination in someone who has already had Prevenar 20), give it under its arm of this PGD; otherwise advise how the vaccine can be accessed through the GP. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Prevenar 20 suspension for injection in pre-filled syringe (pneumococcal polysaccharide conjugate vaccine, 20-valent, adsorbed), 0.5 mL.
Legal category	POM
Storage	Store in a refrigerator (2 to 8 C) in the original packaging. Do not freeze; discard if frozen. Store pre-filled syringes horizontally. Use as soon as possible after removal from the refrigerator. Any excursion is quarantined and assessed under the cold chain page of this PGD; the SmPC stability data for that assessment are 96 hours at 8 to 25 C or 72 hours at 0 to 2 C, and the vaccine is returned to 2 to 8 C and the excursion recorded.
Route/method of administration	Intramuscular injection only, into the deltoid muscle of the upper arm. Shake vigorously to a homogeneous white suspension and do not use if it cannot be resuspended or shows particles or discolouration. Do not give intravascularly. Not for subcutaneous use.
Dose and frequency	One 0.5 mL dose, once, at any age from 2 years. Under this PGD the dose is given at least 8 weeks after any previous pneumococcal conjugate vaccine at any age (the SmPC states this interval for 2 to 17 years; this PGD applies it to adults as well). Not repeated under this PGD: for asplenia, splenic dysfunction or chronic kidney disease (nephrotic syndrome, CKD stage 4 or 5, dialysis or kidney transplant) the later 5-yearly revaccination cycles are Pneumovax 23.
Quantity to be administered and/or supplied	0.5 mL per dose: one pre-filled syringe.
Maximum or minimum treatment period	A single lifetime dose under this PGD. Infant and toddler doses, repeat doses, and the multi-dose sequences for severe immunocompromise

	and bone marrow transplant are not given under this PGD.
Adverse effects	Adults, very common: injection site pain, muscle pain, joint pain, fatigue, headache; common: fever, injection site redness or swelling; uncommon: hypersensitivity including facial swelling and angioedema, urticaria, lymphadenopathy, chills, diarrhoea, nausea, vomiting. Children and adolescents 5 to 17 years, very common: injection site reactions, headache, muscle pain, fatigue; common: joint pain; uncommon: urticaria, fever. Children 2 to 4 years, very common: injection site reactions, irritability, drowsiness, decreased appetite, fever. Anaphylaxis has been reported with pneumococcal conjugate vaccines. Refer to the SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date, product name and brand, batch number, expiry date, dose, route and site; the eligibility group relied on and the pneumococcal vaccine history checked ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Seek medical advice for a severe or persistent injection site reaction, a fever that does not settle within 48 hours, or any sign of an allergic reaction. Call 999 for difficulty breathing, swelling of the face or throat, or collapse. This is a single dose: adults at 65 and most risk groups need no further pneumococcal vaccine. Asplenia, splenic dysfunction and chronic kidney disease have a repeat dose

	every 5 years, given with Pneumovax 23 (Prevenar 20 is given once only) or through the GP.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 25 Pneumococcal, edition of 29 July 2026; JCVI advice on the pneumococcal programme, June 2023 and June 2024; Summaries of Product Characteristics for Prevenar 20 (emc 13461) and Pneumovax 23, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 008

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
008	Prevenar 20 arm added to this PGD, replacing the Prevenar 13 arm	24 September 2026

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v009

Issued: 30 September 2026

Supersedes: Version 008, 24 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v008

Issued: 24 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 24 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

A Prevenar 20 (PCV20) arm is added, with its own criteria, medicine details and follow-up pages after the Pneumovax 23 arm. Prevenar 20 is the product the national programme has used for adults at 65 and for the clinical risk groups since early 2026 and PPV23 can no longer be ordered through the programme; a customer reported on 24 September 2026 that the document still said Prevenar 20 was outside its scope, a gap on the review register since version 003. Written from the Prevenar 20 SmPC (emc 13461: licensed from 6 weeks; one 0.5 mL dose from 2 years, with an 8 week interval after any previous conjugate vaccine stated for 2 to 17 years and applied to all ages by this PGD; need for revaccination not established, so no repeat dose is authorised; contraindicated with hypersensitivity to diphtheria toxoid; intramuscular only; caution in bleeding disorders, immunocompromise and pregnancy; cold chain 2 to 8 C with the SmPC excursion data) and Green Book chapter 25 as summarised in part 2.

The Prevenar 13 arm is withdrawn. The programme gives no further PCV13 from 2 years of age and Prevenar 20 covers every group this PGD serves, so the arm had no defined population once Prevenar 20 was authorised. The cover title, the part 2 note, the schedule table and the references now name Prevenar 20 and Pneumovax 23. Decision of the Medical Director, 24 September 2026, on the hostile review of the draft.

The Pneumovax 23 arm is aligned with the Prevenar 20 arm and with part 2: the eligible groups are named; Prevenar 20 is the arm to use where it is held and Pneumovax 23 runs down existing stock, serves the subcutaneous route and gives the 5-yearly revaccination of asplenia, splenic dysfunction and chronic kidney disease; any previous PPV23 or PCV20 excludes except that revaccination, matching the Green Book statement that a person vaccinated for a risk group needs no dose at 65; an 8 week interval after a conjugate vaccine, the same fever wording, and referrals for severe immunocompromise and for current or recent chemotherapy or radiotherapy are stated in both arms; the 'never within 3 years' residue and the 'further dose of the course' follow-up line are removed.

Severe immunocompromise (bone marrow transplant, acute or chronic leukaemia, multiple myeloma, genetic immune disorders) is a referral in both arms: the Green Book sequence for that group is more than one dose and needs specialist input, and version 007 already said so in the schedule table while the arms still admitted the group. Individuals outside the eligible groups, such as a healthy adult under 65 asking privately, are stated to be outside this PGD. The NICE MPG2 reference address is corrected and the references name the Green Book edition and the SmPCs. The consultation tool offers Prevenar 20 and Pneumovax 23 with the same rules and no longer offers Prevenar 13.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 10: the guidance summary was rewritten against Green Book chapter 25 of 29 July 2026 (Prevenar 20 in the adult and at-risk programmes from early 2026 with PPV23 stock used up first, Capvaxive licensed but not in the programme, infant PCV13 at 16 weeks and 1 year, the Table 2 clinical risk groups including occupational metal fume exposure and people experiencing homelessness, no further PCV13 from 2 years of age, PCV20 for severe immunocompromise, 5-yearly revaccination only for asplenia, splenic dysfunction and chronic kidney disease, contraindications, timing around splenectomy and immunosuppressive treatment, pregnancy, side effects by product and co-administration); the Pneumovax 23 and Prevenar 13 arms are unchanged and the summary states that Prevenar 20 is not authorised under this PGD and that the Prevenar 13 arm keeps its 8 week interval before PPV23.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary now carries a note that it describes the national programme (Prevenar 20 from July 2026, and the infant schedule) while this PGD authorises Prevenar 13 and Pneumovax 23 only, from 2 years of age.

The Pneumovax 23 exclusion for a dose within the last 5 years no longer carries the circular 'unless 5 years have elapsed' exception; revaccination is stated as the dose row has it, for asplenia, splenic dysfunction or chronic kidney disease only and only after 5 years.

Both cautions rows now ask for the batch number, expiry date and site; previously one arm asked for batch and expiry and the other for batch and site.

The Prevenar 13 maximum treatment period row now says a single dose under this PGD instead of 'including primary and booster doses', which the inclusion criteria exclude.

The follow-up advice rows, which carried the treatment template about 3 to 4 weeks, now give vaccination follow-up advice.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name Green Book chapter 25, JCVI and the SmPCs as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.

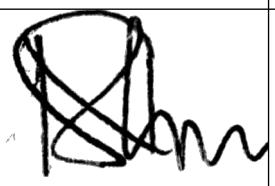
Pneumovax 23: exclusion added for PPV23 or PCV20 received within the last 5 years. The SmPC says revaccination within 3 years is not recommended and the Green Book restricts 5-yearly revaccination to asplenia, splenic dysfunction and chronic kidney disease. The vague 'may be considered' line is replaced with that rule.

Prevenar 13: hypersensitivity to diphtheria toxoid (the CRM197 carrier) added as an SmPC contraindication. Inclusion restricted to 2 years and over; infant primary immunisation is not given under this PGD and 'according to the national immunisation schedule' is replaced by a stated dose.

The guidance summary has not yet been rewritten against the July 2026 Green Book chapter (Prevenar 20, infant dose at 16 weeks). That is recorded in the review register for the next version.

Signed on behalf of Get Real Health

Version v009, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.